PubMed Literature Digest

October 28, 2026 | AI, ultrasound and maternal health

PDF copy of the digest already emailed today. No additional, previously undelivered papers were found on rechecking. The three papers below are reproduced for your requested PDF copy, not counted as new discoveries.

Selection: RSS entry dates within the past seven days, October 22-28, 2026. Entry dates are distinct from article publication dates. Summaries draw on abstracts included in the RSS XML downloaded and parsed directly with Python.

1. Beyond Mallampati: From Universal Bedside Tests to Precision Airway Risk Stratification

A narrative review proposes individualized airway assessment combining ultrasound and machine learning, emphasizing validation, calibration and equity before routine adoption.

RSS entry: October 26, 2026 | Published: October 16, 2026

https://pubmed.ncbi.nlm.nih.gov/42930806/

2. Ensuring Safe and Responsible Use of Artificial Intelligence in Medical Imaging: Pitfalls and Practical Solutions through the Lens of Obstetric Ultrasound

This review distinguishes assistive, diagnostic, predictive and prognostic AI, highlighting data leakage, bias and inadequate external validation in obstetric imaging.

RSS entry: October 24, 2026 | Published: September 29, 2026; October collection

https://pubmed.ncbi.nlm.nih.gov/42921404/

3. Performance of AI-based models in prenatal detection of trisomy 21 via ultrasonography: a systematic review

Across 19 studies, reported performance was promising but predominantly retrospective, with high bias risk in 10 of 15 prediction-model studies and no demonstrated clinical screening benefit.

RSS entry and online publication: October 23, 2026

https://pubmed.ncbi.nlm.nih.gov/42909530/

Connections to your research and practice

AI-POCUS education: Use the obstetric review to teach AI capabilities, failure modes and the distinction between prediction and diagnosis.

Maternal-health study design: The trisomy 21 review supports prospective multicentre validation and patient-level separation of training and test data in your protocols.

Responsible AI and technology assessment: Evaluate subgroup fairness, workflow and patient outcomes alongside accuracy when assessing tools for underserved settings. These are implications for your work, not demonstrated findings in Kenyan settings.